

YOUR CONDITION · TENDRA

What is actually going on in your lower back.

An honest look at why lower back pain happens, why it is almost never as bad as it feels, and what actually moves the needle.

A short read · About 4 minutes · **For anyone with a back that does not feel right**

It is 11pm. Something tweaked in the lower back earlier today, and now Google is involved. The search history goes: lower back pain → causes → bulging disc → degenerative disc disease → sciatica → spinal fusion. By the time the lights go off, the body has been mentally upgraded to a code-red structural disaster.

Most of that is not what is happening.

01 Almost everyone has been here

Roughly four out of five adults will deal with significant lower back pain at some point in their lives. It is the second most common reason people see a doctor, right after the common cold. If lower back pain were a sign that something serious was wrong, the species would have died out a long time ago.

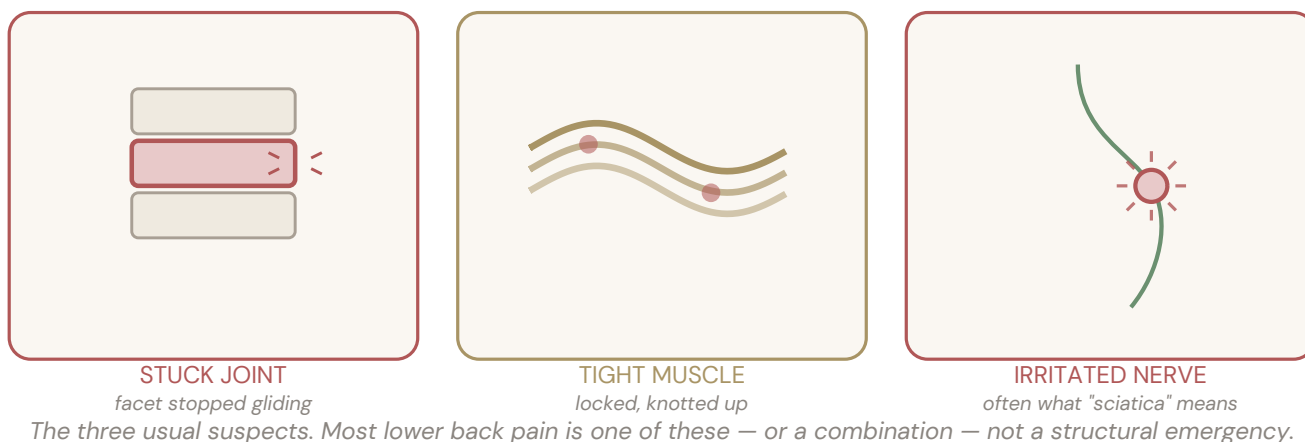
What it actually is, in the overwhelming majority of cases, is a *mechanical* issue. A joint that stopped moving the way it should. A muscle that has been clenched for two weeks straight without being told to relax. A small irritation in a region that has more moving parts than most people realize. None of these are emergencies. All of them respond to care.

This article is about the 95% of lower back pain that is mechanical. The remaining 5% — the part that warrants a different conversation — is covered at the end.

02 What is actually down there

The lower back is doing a lot of jobs at once. Five large vertebrae stack to support the upper body. Between each pair is a disc that absorbs load and lets the spine move. Behind those discs sit small joints called facet joints — about the size of a fingernail — that guide motion. Layered on top of all of that are deep stabilizing muscles, larger movement muscles, and a web of nerves that runs from the spine out into the legs.

When lower back pain shows up, it is almost always one of three things:



A joint stopped gliding properly. The facet joint got stuck or restricted. It is small, but its protest is loud.

A muscle is locked down. Often as a protective response to the joint above. Muscles are not subtle when they are tired of holding something steady.

A nerve is being irritated. Usually mechanically — by an inflamed joint or a tight muscle, not by structural damage. This is what most people mean when they say "my sciatica is flaring."

03 Why MRIs scare people more than they should

This is the part nobody is told. When researchers MRI the lower backs of pain-free adults — people walking around with zero symptoms — they find "abnormalities" in roughly 60% of them. Disc bulges. Degeneration. Mild facet arthritis. Findings that absolutely come back on a report, and absolutely make a person nervous if they do not know better.

What that tells us: those findings are normal. They are signs of being a human who has used a body for a few decades. They do not, by themselves, cause pain. Plenty of people have them and feel great. Plenty of people have completely clean MRIs and feel terrible.

IN PLAIN TERMS

An imaging report is information, not a diagnosis. Pain has to be matched to the body that is in pain — what aggravates it, what relieves it, how it moves — not to a picture in isolation.

04 What actually helps

The instinct, especially with a brand-new flare, is to lie down. To stop moving until things calm down. This is one of the few things research has been consistent about for thirty years: prolonged bed rest makes mechanical low back pain worse, not better.

Walking. Five to ten minute walks, several times a day. Walking is the single most underrated tool for a lower back that is acting up.

Targeted motion. The exercises prescribed in this app are picked specifically for what is irritated and what needs to wake back up. They are short on purpose. The point is repetition, not heroism.

Heat for muscles, ice for sharp pain. Twenty minutes at a time.

Sleep. Side-lying with a pillow between the knees is the gentlest position for most lower backs. The nervous system does most of its repair work overnight.

Time is also a real ingredient. Most mechanical lower back pain improves substantially within two to four weeks of consistent care and movement. The patients who do worst are usually the ones who froze in fear of moving wrong — not the ones who moved too much.

Most lower back pain is mechanical, not structural — a stuck joint or a tired muscle, not a broken spine. The body is far more resilient than the internet at 11pm would have you believe.

05 When to skip everything above and call someone

CALL THE OFFICE OR SEEK CARE IF YOU NOTICE

- **Numbness in the groin or saddle area**, or new trouble controlling the bladder or bowels.
- **Progressive weakness in a leg** — not a brief tingle, but actually being unable to lift the foot or push off it normally.
- **Severe back pain at night** that is unrelated to position and not tied to a recent injury.
- **Back pain with a fever**, or in someone with a history of cancer.
- **A sudden, severe injury** — a fall, a car accident — that produced new severe pain.

None of these are reasons to panic. They are reasons to get someone looking sooner rather than later. For everything else: keep moving, do the practice, and give the body the time it is designed to use.

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About this guide. This is general patient education and is not medical advice for any specific person. Lower back pain is common but varies a great deal between individuals — any concerns about a specific symptom should go directly to the treating chiropractor or a relevant medical provider. If symptoms feel like a medical emergency, call 911 or go to the nearest emergency department.